

# Trauma Blunt

## Key Concepts

- Survival is rare from traumatic arrest from a blunt mechanism of injury such as a long fall, motor vehicle collision or entrapment
- The focus of care is to correct reversible causes of shock using the **MARCHES** protocol.
- A rapid assessment and initial care should be done on scene with a rapid load process only if pulses return after interventions.
- The Medical Director on call should be contacted very early for termination of resuscitation if indicated.

## MARCHES Protocol

- **Massive bleeding control** - tourniquet/wound packing
- **Airway** – NPA/OPA/Cricothyrotomy
- **Respiratory** – decompress chest if tension pneumothorax suspected, occlusive dressing for open pneumothoraces
- **Circulation**- IV/IO, pelvic binder, reassess tourniquet/ wound packing
- **Hypothermia care/Hike** vs Helicopter (consider evac method)
- **Eye injuries** – cover with rigid shield and no pressure on the eye
- **Spinal motion restriction** if indicated

## EMT/AEMT

- Stop **Massive Bleeding** as first action
  - Tourniquet
  - Compress large vessel in the wound by hand against bone
  - Wound packing
  - Pressure dressing with firm direct pressure
- Establish Simple **Airway** with NPA or OPA and suction secretions if needed
- Support **Respirations**
  - BVM with Oxygen if needed
- Support **Circulation**
  - Start CPR if in arrest
  - Reassess tourniquets, bleeding wounds
  - Place on Monitor or AED

## EMT/AEMT - Expanded Scope

- **TXA** per TXA Guideline
- **NS/LR** according to Trauma General Patient Care Guidelines

## Paramedic

- Assess BLS **MARCHES** interventions
  - If in arrest or profound shock needle thoracostomy on affected side or bilateral if indicated
    - 4-5th intercostal space, anterior axillary line (nipple level), above rib is preferred location
    - 2nd intercostal space, mid-clavicular is alternative location
- Start IV/IO and administer fluids according to Trauma General Patient Care Guideline
- **Tranexamic acid (TXA)** as indicated per TXA Guideline
- If in arrest and no response, consider termination of efforts with On Call Medical Director
- If return of pulses or signs of life, initiate rapid load with transport

**\*\*Continuously monitor tourniquet and wound packing interventions\*\***

**Reassess after every patient move**